

RESTRAINT AND SECLUSION POLICY

Least Restrictive, Last Resort — Authorization, Prohibited Practices, Documentation, Monitoring, and Mandatory Reporting

Document Reference	ECC-BSP-2026-001 (module) / ECC-RS-POL-2026-001
Version / Effective Date	1.0 — July 2026
Approved By	Executive Director
Review Date	July 1, 2027 (annual); also upon any restraint/seclusion incident, funder directive, or legislative change
Applies To	All Evergreen Community Care residential programs and 1–2 bed homes — MCFD (children & youth) stream; all Youth Care Workers/Community Support Workers, Shift Leads, Program Coordinators, Program Managers, and the Executive Director
Legislative / Standards Basis	MCFD SHSS Standard D.5 (SHSS Oct 2016 amendment); Workers Compensation Act (WCA); BC Human Rights Code; Child, Family and Community Service Act (CFCSA); Residential Care Regulation (RCR) s.73–80; CLBC Behaviour Support and Safety Planning Policy (2024) and accompanying Guide for Service Providers
Related Documents	ECC-BSP-2026-001 (Behaviour Support Policy & Procedure Manual); ECC-IIME-2026-001 (Critical Incident Policy); ECC-HC-2026-001 (Health Care & Consent Policy Manual); ECC-OHS-POL-2026-002 (OHS Program Binder); ECC-PC-MAN-2026-001 (Individual Service Plan Process Manual)

1. Policy Statement

Restraint and seclusion are measures of absolute last resort at Evergreen Community Care, used only to prevent immediate, serious physical harm to the person or others, and only after less restrictive strategies have been tried or are clearly insufficient for the immediacy of the risk. They are never used as punishment, discipline, coercion, convenience, or a substitute for adequate staffing, supervision, or a properly developed behaviour support plan.

This policy applies within the least-intrusive intervention hierarchy set out in the Behaviour Support Policy & Procedure Manual (ECC-BSP-2026-001): prevention and environmental strategies, then de-escalation and relational strategies, then individualized behaviour support strategies, and only then — as a last resort — a restrictive intervention governed by this policy.

Every use of restraint or seclusion is treated as a significant event: it is authorized only within defined limits, documented in full, reviewed by the Program Manager, reported to the funder where required, and used as evidence to reduce the likelihood of recurrence — never simply logged and repeated.

2. Definitions

Restraint	Any manual, mechanical, or chemical method that restricts a person's freedom of movement, physical activity, or normal access to their body, used to manage behaviour that presents an immediate risk of serious physical harm
Manual/physical restraint	The use of bodily force by staff to limit or control a person's movement; permitted at Evergreen only as a last-resort safety intervention by staff trained in an approved physical intervention curriculum
Mechanical restraint	Use of a device or equipment to restrict movement; NOT permitted at Evergreen under any circumstance (see Section 4)
Chemical restraint	Use of a medication to control behaviour rather than to treat a diagnosed condition; NOT permitted at Evergreen — see Section 4 and ECC-MED-2026-001 §22
Seclusion	Placing a person alone in a room or area and preventing them from leaving, whether or not the door is locked
Least restrictive intervention	The intervention that resolves the immediate safety risk while limiting the person's rights, movement, and dignity as little as possible, for as short a time as possible
Emergency safety intervention	A restraint or seclusion used in response to an unplanned, immediate risk, as distinct from a planned intervention named in advance in a person's safety plan

3. When Restraint or Seclusion May Be Used

3.1 Threshold for use

Immediate risk required	Used only where there is an immediate risk of serious physical harm to the person or another person — never for property damage alone, non-compliance, verbal aggression without physical risk, or staff convenience
Less restrictive options exhausted	Used only after prevention, environmental, de-escalation, and individualized behaviour support strategies have been tried or are clearly insufficient given the immediacy of the risk
Shortest duration necessary	Ended at the earliest possible moment the immediate risk has passed — never continued after the person has calmed or the risk has resolved
Named in a safety plan where foreseeable	Where a person's presentation makes restraint or seclusion a foreseeable possibility, this is addressed in advance in their Individual Service Plan and behaviour support/safety plan — including specific triggers, preferred de-escalation approaches, and what the least restrictive response looks like for that person

3.2 Authorization

Staff authorization	Only staff trained in an approved physical intervention curriculum (see Section 7) may use manual restraint; untrained staff never physically restrain, regardless of the situation — they call for trained support and prioritize safety by other means
Program Manager notification	The PM is notified immediately (same shift) of any restraint or seclusion use, whether or not it was named in advance in a safety plan
Planned intervention approval	Any restraint or seclusion strategy named in advance in a safety plan is approved in writing by the PM and, for MCFD youth, communicated to the social worker before being added to the plan

4. Prohibited Practices

The following are prohibited at Evergreen Community Care without exception, consistent with the BC Human Rights Code, WCA, and CFCSA:

Mechanical restraint	No device, equipment, or object is used to restrict a person's movement under any circumstance
Chemical restraint	No medication is administered for the purpose of controlling behaviour rather than treating a diagnosed condition under a prescriber's care (ECC-MED-2026-001 §22 governs all psychotropic medication use)
Prone (face-down) restraint	Never used, given the documented risk of positional asphyxia
Restraint or seclusion as punishment	Never used for discipline, retaliation, convenience, or in response to non-dangerous behaviour
Restraint or seclusion involving the mouth, nose, or airway	Never used under any circumstance
Locked seclusion	A person is never confined to a locked room; if a door happens to be closed for safety, a staff member remains immediately present and the door is never locked
Withholding basic needs during any intervention	Food, water, washroom access, and required medication are never withheld or delayed as part of a restraint or seclusion response

5. During an Intervention

Continuous supervision	A person in restraint or seclusion is continuously, directly supervised by a staff member for the entire duration — never left alone
Ongoing risk reassessment	The intervention is reassessed continuously; it ends the moment the immediate risk has passed, regardless of elapsed time
Dignity and communication	Staff speak calmly and respectfully throughout, explain what is happening and why, and support the person to regain calm and control as quickly as possible
Health monitoring	Staff watch for signs of physical distress (breathing difficulty, loss of colour, injury) throughout; any sign of physical harm triggers immediate release and first aid/911 per ECC-IIME-2026-001
Debrief with the person	Once calm, staff debrief with the person in a manner appropriate to their age and understanding — what happened, why, and what could help next time — never skipped because the moment has passed

6. Documentation and Reporting

Same-shift documentation	Every restraint or seclusion event is documented before the end of the shift: what preceded it, what less restrictive strategies were tried, the intervention used, its duration, staff involved, and the person's condition throughout and after
PM same-shift notification	The PM is notified the same shift for every use, whether planned or emergency
Funder reportable incident	Restraint and seclusion use is a reportable circumstance under MCFD SHSS and is reported per the Critical Incident Policy and Reporting Manual (ECC-IIME-2026-001): verbal notification to the funder the same day, written notification within 24 hours
Consent authority notified	The person's consent authority/social worker is notified the same day, consistent with the Health Care & Consent Policy Manual notification standards
PM review within 5 business days	The PM conducts a root-cause review within 5 business days: was the intervention truly least restrictive, was it in the safety plan, what less restrictive strategy could work next time — mirroring the root-cause approach used across our Medication Administration and Critical Incident processes
Pattern review	All restraint/seclusion incidents are aggregated and reviewed at least quarterly by the PM/ED to identify patterns — a repeated need for restraint with one person triggers an urgent behaviour support plan review, not routine logging

7. Training Requirements

Training	Who	When	Renewal
Approved physical intervention curriculum (e.g., NVCI or equivalent)	All staff who may use manual restraint	Before authorization to use manual restraint; never used untrained	Annually
De-escalation and behaviour support training	All direct support staff	At orientation, before unsupervised duties	Annually
This policy — read and acknowledged	All staff	At orientation	On each annual re-issue

Training currency is tracked on the monthly staff-currency check alongside first aid and MALM; a staff member whose physical intervention training has lapsed is not authorized to use manual restraint until retrained, regardless of experience.

8. Roles and Responsibilities

All staff	Use restraint or seclusion only if trained and only as an absolute last resort; document the same shift; never use a prohibited practice under any circumstance
PC	Ensures every applicable Individual Service Plan addresses foreseeable safety risk with a least-restrictive-first approach; monitors documentation completeness
PM	Notified same-shift of every use; conducts the 5-business-day root-cause review; approves any planned intervention in advance; reviews patterns quarterly; ensures training currency
ED	Approves this policy annually; ensures funder reporting occurs per timeline; responds to any systemic pattern identified

9. Compliance Calendar

Task	DUE DATE	DATE DONE	INITIALS
Physical intervention training — new staff before authorization	Before first use		
Training currency check (with MALM/first aid)	Monthly, by the 5th		
Same-shift documentation of any restraint/seclusion use	Same shift		
Funder verbal notification	Same day		
Funder written notification	Within 24 hours		
PM root-cause review	Within 5 business days		
Pattern review across all incidents	Quarterly		
Policy review and ED re-approval	July 1, 2027		

Approval & Staff Acknowledgement

I have read and understood the Restraint and Seclusion Policy (ECC-RS-POL-2026-001). I understand that restraint and seclusion are last-resort measures only, I understand the prohibited practices in Section 4, and I will not use manual restraint unless I am currently trained and authorized to do so.

Executive Director — Signature / Date	
Program Manager — Signature / Date	
Staff Name (print) / Signature / Date	
Physical intervention training completed (date / curriculum)	